

Policy on Continuous and Multi-Disciplinary Patient Care

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» ensures patient care is continuous and multi-disciplinary: a named responsible doctor at all times, coordinated care across settings, shared clinical information, standardised hand-over communication, safe internal transfer, written referral guidance, predictable service delivery, and monitoring of the response to critical value alerts.

1. Purpose

This policy explains how «Hospital Name» maintains continuity and coordination of patient care across all care settings: naming a responsible doctor, coordinating care, sharing clinical information among providers, standardising hand-over, transferring patients safely, referring within the organisation, communicating schedule changes, and monitoring the response to critical value alerts.

Multi-disciplinary care-plan discussion and transfer to another facility are covered in the hospital's other policies.

2. Scope

This policy applies to all doctors and nurses providing direct patient care in out-patient, emergency, in-patient, intensive care, and any other care setting at «Hospital Name».

3. Policy standards

«Hospital Name» identifies a qualified individual responsible for each patient's care during all phases of care, coordinates care among providers in each setting, and shares information about care and response to treatment among medical, nursing and other care providers.

Standardised hand-over communication is implemented during shifts, between shifts, and during transfers; patient transfer within the organisation is done safely; referral to other departments follows written guidance; service delivery is predictable and schedule changes are communicated; and a mechanism monitors the clinical response to critical value alerts.

4. Non-negotiable rules

- Do not leave a patient without a named responsible doctor in any phase or setting of care.
- Do not allow a hand-over between shifts, units or departments to proceed undocumented, or without acknowledgement by the receiving clinician.
- Do not transfer a patient within the organisation without documented hand-over and takeover, or without defined accompaniment and monitoring for an unstable patient.
- Do not refer a patient to another department or speciality outside the organisation's written referral guidance on grades, timeframes and who may accept a referral.
- Do not leave a critical value alert without a documented, monitored clinical response.

5. What we do

5.1 Name a responsible doctor for every patient

During all phases of care, a qualified doctor is identified as responsible for the patient's care. The responsible doctor's name is recorded in the patient record at first contact and updated when responsibility changes. When the named doctor is off duty, the covering doctor is identified and the change is recorded.

5.2 Coordinate care across settings

Patient care is coordinated among care providers in each care setting — out-patient, emergency, in-patient, intensive care, and any other setting the organisation uses. The patient's requirements are communicated to providers in the receiving setting so that orders, observations and pending tasks are not lost when the patient moves.

5.3 Share care and response information among providers

Information about the patient's care and response to treatment is shared among medical, nursing and other care providers through entries in the case sheet or electronic medical record. The patient record remains available to authorised providers.

5.4 Implement standardised hand-over communication

Standardised hand-over communication is implemented during each staffing shift, between shifts, and during transfers between units or departments. Hand-over by doctors and nurses in

direct patient care is documented and covers the patient's current condition, recent changes, ongoing treatment, and possible changes or complications. The receiving clinician acknowledges the hand-over, and interruptions during hand-over are minimised.

5.5 Refer within the organisation following written guidance

Referral to other departments or specialities within the organisation follows written guidance. Referral may be for opinion, co-management or takeover, and the referral note states the reason. Urgency is graded, and every referral is seen within a defined timeframe per grade. Written guidance covers grades, timeframes and who may accept a referral.

5.6 Transfer patients within the organisation safely

Patients are transported in a safe and timely manner within the organisation. Proper hand-over and takeover are documented. Transfers to and from imaging follow the same rule so that imaging is not delayed by transport failure. Unstable patients are transferred with the accompaniment and monitoring defined for their clinical status.

5.7 Ensure predictable service delivery

The organisation ensures predictable service delivery by adhering to defined timelines for services such as laboratory, radiology, and OPD waiting time. Patients are informed of the applicable timelines. When delivery will deviate, the patient, family and/or caregiver is informed of the changed schedule, and the service area records that the change was communicated.

5.8 Monitor response to critical value alerts

A mechanism monitors whether an adequate clinical intervention has taken place in response to a critical value alert. The attending clinician's response is documented in progress notes or medication orders. Interventions are periodically reviewed for timeliness and appropriateness. For out-patients, efforts are taken to alert the patient or family about critical values.

6. Stop-work authority

Not applicable. This standard has no stop-work authority. Gaps in continuity of care — a missing responsible doctor, an undocumented hand-over, an uncoordinated transfer, or an unmonitored critical value response — are addressed through the hospital's quality monitoring and CAPA process (Section 8), not through an immediate stop-work trigger.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for continuity and coordination of patient care across all settings.
Treating/Responsible Doctor	Named as responsible for the patient's care; ensures hand-over and referral guidance is followed; responds to critical value alerts.
Nursing Superintendent	Ensures standardised hand-over and safe

	internal transfer practice among nursing staff.
Quality Coordinator	Audits this policy; holds hand-over, referral, transfer, and critical-value-response records.

8. Quality monitoring

Monitoring area	What is monitored
Responsible doctor	Patient record entries naming the responsible doctor across care settings; covering-doctor records.
Care coordination	Communication or transfer notes when patients move across care settings.
Information sharing	Case sheet or EMR entries by multiple provider types on the same patient.
Hand-over	Documented hand-over records for each staffing shift and transfer, with receiving-clinician acknowledgement.
Internal referral	Referral notes with reason and grade; response-time log against defined timeframes.
Internal transfer	Internal transfer records with hand-over/takeover; unstable-patient accompaniment and monitoring records.
Schedule predictability	Defined service timelines; records of schedule-change communication to patient/family/caregiver.
Critical value response	Critical value alert log; documented clinical intervention; periodic review of timeliness and appropriateness.

This policy is reviewed annually, or sooner if NABH standards change.

9. Training and staff acknowledgement

All doctors and nurses providing direct patient care shall be trained on the responsible-doctor rule, standardised hand-over, safe internal transfer, and internal referral guidance in this policy, at induction and periodically.

I have read the Policy on Continuous and Multi-Disciplinary Patient Care of «Hospital Name». I will follow the processes described.

10. Distribution

This policy shall be available to all clinical and nursing staff, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
EMR	Electronic Medical Record
ICU	Intensive Care Unit
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OPD	Out-Patient Department

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
AAC.10.a	Commitment	Sections 3 and 5.1 address naming a responsible doctor for all phases of care.
AAC.10.b	Commitment	Sections 3 and 5.2 address coordination of care across settings.
AAC.10.c	Commitment	Sections 3 and 5.3 address sharing of care and response information among providers.
AAC.10.d	CORE	Sections 3 and 5.4 address standardised hand-over communication.
AAC.10.e	Commitment	Sections 3 and 5.6 address safe internal patient transfer.
AAC.10.f	Commitment*	Section 5.5 addresses written referral guidance within the organisation.
AAC.10.g	Commitment	Sections 3 and 5.7 address predictable service delivery and schedule-change communication.
AAC.10.h	Commitment	Sections 3 and 5.8 address monitoring the response to critical value alerts.

13. Required records/evidence checklist

Responsible doctor — AAC.10.a (Commitment)

- Patient record entry naming the responsible doctor for each phase of care.
- Covering-doctor record when the named doctor is off duty.

Care coordination — AAC.10.b (Commitment)

- Communication or transfer notes when patients move across care settings.

Information sharing — AAC.10.c (Commitment)

- Case sheet or EMR entries by multiple provider types on the same patient.
- Record of team meetings where used.

Hand-over — AAC.10.d (CORE)

- Documented hand-over records using the standardised framework for each staffing shift.
- Transfer hand-over with acknowledgement by the receiving team.

Internal transfer — AAC.10.e (Commitment)

- Internal transfer record with reason, clinical status, times, hand-over and takeover.
- Imaging transfer records; unstable-patient transfer records with accompaniment and monitoring.

Internal referral guidance — AAC.10.f (Commitment*)

- Current written guidance for internal referral (opinion/co-management/takeover; urgency grades; timeframes).
- Referral notes with reason and grade; response-time log.

Predictable service delivery — AAC.10.g (Commitment)

- Defined service timelines; record that patients were informed of timelines.
- Record of schedule-change communication to patient/family/caregiver.

Critical value response — AAC.10.h (Commitment)

- Critical value alert log with time and clinician notified.
- Progress notes or orders documenting the clinical intervention.
- Periodic review records of response timeliness and appropriateness.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. AAC.10.

Disclaimer

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